

पी० एस० ई० (हिन्दी)
PRESENT STATE EXAMINATION
(NINTH EDITION, 1974)
(Including Hindi Adaptation 1978)

WHO COLLABORATING CENTRE FOR TRAINING AND RESEARCH IN
MENTAL HEALTH, DEPARTMENT OF PSYCHIATRY,
POSTGRADUATE INSTITUTE OF MEDICAL EDUCATION AND RESEARCH,
CHANDIGARH-160 012, INDIA.

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ADDRESS : _____ DATE OF REGISTRATION : _____

FIELD RESEARCH CENTRE :

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PROJECT NO : _____

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NAME :

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DAY	MONTH	YEAR

NAME :

--	--	--

TIME TAKEN TO COMPLETE THE INTERVIEW
(IN MINUTES): _____

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NO. OF SESSIONS REQUIRED TO COMPLETE
THIS PSE INTERVIEW,
SPECIFY WITH DATES :

SESSION NO.	DATE	SECTIONS RATED
1		
2		
3		

PLACE OF INTERVIEW : _____ HAS THIS PATIENT BEEN INTERVIEWED WITH PSE
EARLIER, SPECIFY PREVIOUS DATES : _____

USE OF CASE RECORDS (SPECIFY) :

INSTRUCTIONS

The instruction manual contains a detailed description of the origins, development and underlying principles of the PSE and a glossary of definitions of symptoms. The examiner must be thoroughly familiar with the manual and glossary and should have had some prior training in the use of the PSE.

Four kinds of questions are written into the schedule :

(a) Obligatory (starred) questions

These must be asked if the interview is conducted at all. Only 54 questions are involved. Thus subjects with no symptoms, who ask clarifying questions of their own and who answer clearly and decisively, can be screened very quickly indeed. Whenever there is any doubt, however, and certainly whenever a symptom needs clarification, the second kind of question should be asked.

(b) Bracketed questions above cut-off points These help to define the nature and extent of a symptom and should always be asked if there is any doubt about replies to obligatory questions.

(c) Unbracketed questions below cut-off points

Once the examiner has proceeded below a cut-off point, he must ask all the unbracketed questions in that part of the section.

(d) Bracketed questions below cut-off points

These serve the same function as similar questions above cut-off points, i.e. they help to define the nature and extent of a symptom. They are used only if there is some other evidence that the symptom is present.

In addition, the examiner himself will usually wish to ask other questions which are not written into the schedule, either general probes or more specific questions, depending on the nature of the patient's replies.

Each symptom is defined to some extent within schedule itself but the examiner must be completely familiar with the fuller definitions in the glossary. A full discussion of scoring is also included in the glossary, particularly as to how to differentiate (0) from (1), and (1) from (2).

(0) = Examiner satisfied that symptom not present to clinically significant degree during past month..

(8) = Examiner not sure whether symptom present during past month, even though the appropriate questions have been asked, and answered without incoherence or evasion. The symptom cannot be excluded.

(9) = No rating can be made because question not asked or subject does not answer or answer is incomprehensible.

It should be emphasised that using the PSE schedule will not in itself guarantee useful results. The quality of the output of any system depends on the quality of the input.

1. INTRODUCTION

The interviewer should introduce himself briefly, describe the purpose of the interview and explain about any recording equipment. The purpose of the introductory section is to obtain an overall picture of the symptomatology, in the subject's own words.

To begin with, I should like to get an idea of the sort of problems that have been troubling you during the past month. What have been the main difficulties?

मैं यह जानना चाहूंगा कि पिछले एक महीने में आपको कौन सी या किस तरह की तकलीफें रही हैं? ज्यादा तकलीफें किन बातों से रही हैं?

Record the main symptoms spontaneously mentioned.

Means of exploration, if subject gives inadequate information :

*If subject's statement too brief
If subject has no more to add
If statements are difficult to understand
If subject is vague
If no other response forthcoming*

Can you tell me more about that?
What else has been troubling you?
Can you explain what you mean by?
Could you give an example of?
Why did you come to the hospital?

RATE PATIENT'S ACCOUNT OF SYMPTOMS.

0=Subjects responds adequately.

1=Accounts somewhat inadequate but interview can proceed.

(see 140)

2=Account seriously inadequate but interview proceeds in an attempt to rate some subjective responses, as well as behaviour, affect and speech.

3=Impossible to continue with interview. Only behaviour, affect and speech sections rated.

REASONS FOR INADEQUACY (TICK AS MANY AS APPROPRIATE).

Denial or guardedness	—	Inattention	—
Incoherence	—	Refusal	—
Irrelevance	—	Patient mute, stuporous, etc.	—
Replies too brief	—	Other, specify	—
Poverty of content of speech	—		

IF (1) OR (2) CARRY ON WITH SECTION 2, UNLESS SUBJECT MENTIONS OR HINTS AT DELUSIONS OR HALLUCINATIONS → SECTION 18.

Cut off

Current treatment, if subject not seen in hospital or clinic.
Rate the following if sufficient information has already emerged. If not, use the suggested question.

MAY I ASK IF YOU ARE SEEING ANY DOCTOR FOR YOUR NERVES?

मन की परेशानी या अपनी घबराहट के लिये क्या आप किसी डाक्टर को दिखा रहे हैं?

Or specify if psychosomatic complaints.

WHAT KIND OF DOCTOR IS HE? YOUR OWN GP? A PRIVATE DOCTOR? PSYCHIATRIST?

कौन से डाक्टर साहब हैं?

- 0 = No doctor
- 1 = GP
- 2 = Private doctor other than GP
- 3 = Psychiatrist
- 4 = Hospital out-patient (other than psychiatric)
- 5 = Other paramedical specialist, or osteopath
- 6 = Other specify

ARE YOU ATTENDING FOR TREATMENT ANY PERSON WHO IS NOT MEDICALLY QUALIFIED, e. g. LAY

THERAPIST, HERBALIST, ACUPUNCTURE FAITH HEALER, CHRISTIAN SCIENCE, CHURCH WHICH FORBIDS MEDICAL ADVICE?

अपने इलाज के लिये क्या आपने किसी भाड़फूंक वाले, हकीम, वैद्य, या भूतप्रेत उतारने वाले को दिखाया है?

WHAT WERE YOU COMPLAINING OF AT THE TIME?

उस समय आपको क्या तकलीफ थी?

Complaint

Specify type of treatment

2. HEALTH, WORRYING, TENSION

**** IS YOUR PHYSICAL HEALTH GOOD?**

क्या आपकी शारीरिक (जिस्मानी) सेहत ठीक रहती है?

(Does your body function normally?)

**** DO YOU FEEL YOU ARE PHYSICALLY ILL IN ANY WAY?**

क्या लगता है कि आपको कोई शारीरिक (जिस्मानी) बीमारी है?

(What is that like? How serious is it?)

RATE SUBJECTS OWN SUBJECTIVE EVALUATION OF PRESENT PHYSICAL HEALTH (irrespective of whether physical disease is present).



(1)

- 0 = Feels physically very fit.
- 1 = Feels particular physical complaint but does not say positively feels fit.
- 2 = Feels unwell but not seriously incapacitated.
- 3 = Feels seriously incapacitated by physical illness.

**** WHAT DOES YOUR DOCTOR SAY IS WRONG?**

डाक्टरों ने बताया कि क्या बीमारी है, क्या नुक्स है?

(Have you had a physical illness recently; colds, influenza, etc?)

RATE PRESENCE OF PHYSICAL ILLNESS OR HANDICAP

taking results of recent investigations and physical state examinations into account.

0 = No physical illness or handicap present.

1 = Mild but significant physical illness or handicap (e.g. influenza or limp).

2 = More serious physical illness or handicap present but not incapacitating of threatening to life (e.g. deafness or duodenal ulcer).

3 = Physical illness or handicap present which is incapacitating or threatening to life (e.g. blindness or carcinoma).

Specify illness, disabilities and duration :

RATE PSYCHOSOMATIC SYMPTOMS.

Special projects only

☐

(2)

☐

(3)

**** HAVE YOU WORRIED A LOT DURING THE PAST MONTH?**

क्या पिछले महीने आपको बहुत ज्यादा चिन्ता रही है?

(What do you worry about?)

PROBE : (Money, housing, children, health, work, marriage, relatives, friends, neighbours, other).

(How much do you worry? Are you a worrier?)

If any indication of worry, use further probes :

**** WHAT IS IT LIKE WHEN YOU WORRY?**

जब चिन्ता होती है तब आपको कैसा महसूस होता है?

(What sort of state of mind do you get into?)

(Do unpleasant thoughts constantly go round and round in your mind?)

(Can you stop them by turning your attention to something else?)

RATE WORRYING : *A round of painful thought which cannot be stopped and is out of proportion to the subject worried about.*

1 = Symptom definitely present during past month, but of moderate clinical intensity or intense less than 50% of the time.

2 = Symptom clinically intense more than 50% of the month.

☐

(4)

**** HAVE YOU HAD HEADACHES, OR OTHER ACHES OR PAINS DURING THE PAST MONTH?**

क्या पिछले महीने आपको सिर दर्द या और किसी तरह का दर्द या पीड़ा हुई है?

(What kind?)

RATE ONLY TENSION PAINS, e.g. 'band round head', 'pressure', 'tightness in scalp', 'ache in back of neck', not migraine.

1 = Symptom definitely present during past month, but of moderate clinical intensity, or intense less than 50% of the time.

2 = Symptom clinically intense more than 50% of past month.

☐

(5)

**** HAVE YOU BEEN GETTING EXHAUSTED AND WORN OUT DURING THE DAY OR EVENING, EVEN WHEN YOU HAVEN'T BEEN WORKING VERY HARD?**

बिना कोई ज्यादा काम किये भी, सुबह या शाम को आप बहुत ज्यादा थकावट (थके थके) महसूस करते हैं?

RATE TIREDNESS OR EXHAUSTION : *Do not include tiredness due to flu, etc.*

1 = Only moderate form of symptom (tiredness) present; or intense form (exhaustion) less than 50% of the time.

2 = Intense form of symptom (exhaustion) present more than 50% of the past month.

☐

(6)

**** HAVE YOU HAD DIFFICULTY IN RELAXING DURING THE PAST MONTH?**

क्या पिछले महीने में आपको सुस्तानें या आराम (रिलैक्स) करने में मुश्किल हुई है?

(Do your muscles feel tensed up?)

RATE MUSCULAR TENSION : *Do not include a subjective feeling of nervous tension, which is rated later.*

- 1 = Symptom definitely present during past month, but of moderate clinical intensity, or intense less than 50% of the time.
2 = Symptom clinically intense more than 50% of past month.

☐ (7)

**** HAVE YOU BEEN SO FIDGETY AND RESTLESS THAT YOU COULDN'T SIT STILL?**

क्या कभी कभी आप इतने बेचैन हो जाते हैं कि आराम से एक जगह टिक कर बैठ नहीं जाता है?

(Do you have to keep pacing up and down)

RATE RESTLESSNESS.

- 1 = Only moderate form of symptom (fidgety, restless) present; or intense form (pacing, can't sit down) less than 50% of the time.
2 = Intense form of symptom (pacing, etc.) present more than 50% of past month.

☐ (8)

**** DO YOU TEND TO WORRY OVER YOUR PHYSICAL HEALTH?**

क्या आप अपनी शारीरिक सेहत के बारे में चिन्ता करते रहते हैं?

RATE HYPOCHONDRIASIS : *Overconcern with possibility of death, disease or malfunction. Re-rate at end of interview if subject constantly reverts to hypochondriacal preoccupation. Consider ratings of symptoms (1) and (3).*

- 1 = Symptom present during past month, but not (2)
2 = Subject constantly reverts to hypochondriacal preoccupations during interview.

☐ (9)

**** DO YOU OFTEN FEEL ON EDGE OR KEYED UP OR MENTALLY TENSE OR STRAINED?**

क्या आप अक्सर परेशान से रहते हैं जैसे कि आप के मन में कई तरह के खिंचाव और तनाव रहते हों?

(Do you generally suffer with your nerves ?)

(Do you suffer from nervous exhaustion?)

RATE SUBJECTIVE FEELING OF NERVOUS TENSION :

There is no need for autonomic accompaniments for this symptom to be rated present.

- 1 = Symptom definitely present during past month, but of moderate intensity, or intense less than 50% of the time.
2 = Intense form of symptom present more than 50% of the past month.

☐ (10)

**** DO YOU FIND THAT A LOT OF NOISE UPSETS YOU?**

क्या आप बहुत ज्यादा शोर होने पर परेशान हो जाते हैं?

(Do noises sometimes seem to penetrate, or go through your head?)

RATE HYPERSENSITIVITY TO NOISE

- 1 = Moderate degree during month.
2 = Severe degree during month.

3 AUTONOMIC ANXIETY

In this section, rate only subjective anxiety with autonomic accompaniments, either free-floating or situational. Do not include worrying or nervous tension. Do not include anxiety due to e.g., persecutory delusions, except in the special item (no. 13).

(CHECK LIST of autonomic accompaniments :

Blushing	Dry month
Butterflies	Giddiness
Choking	Palpitations
Difficulty getting breath	Sweating
Dizziness	Trembling)

** HAVE THERE BEEN TIMES LATELY WHEN YOU HAVE BEEN VERY ANXIOUS OR FRIGHTENED?

क्या पिछले दिनों कभी ऐसा हुआ कि आप बहुत घबरा गये हों या डर गये हों?

(What was this like?)

(Did your heart beat fast?) Ask for other autonomic symptoms.

(How often in the past month?)

RATE FREE-FLOATING AUTONOMIC ANXIETY : *Exclude if due to delusions. Exclude if purely situational.*

☐ (11)

1 = Symptom definitely present, with autonomic accompaniment, during past month, but of moderate clinical intensity, or intense less than 50% of the time.

2 = Symptom clinically intense more than 50% of the time.

** HAVE YOU HAD THE FEELING THAT SOMETHING TERRIBLE MIGHT HAPPEN?

क्या आपको कभी ऐसा लगा कि जैसे कोई बहुत ही बुरी या खराब बात होने वाली है?

(That some disaster might occur but you are not sure what? Like illness or death or ruination?)

(Have you been anxious about getting up in the morning because you are afraid to face the day?)

(What did it feel like?)

RATE ANXIOUS FOREBODING WITH AUTONOMIC ACCOMPANIMENTS.

☐ (12)

1 = Symptom definitely present, with autonomic accompaniment, during past month, but of moderate clinical intensity, or intense less than 50% of the time.

2 = Symptom clinically intense more than 50% of the time.

RATE AUTONOMIC ANXIETY DUE TO DELUSIONS, etc. and if necessary defer to end of interview.

☐ (13)

0 = No anxiety due to delusions or hallucinations.

1 = Subject complains of anxiety but no evidence of anxiety on examination.

2 = Clearly anxious or frightened because of delusions or hallucinations.

CUT OFF IF NO EVIDENCE OF ANXIETY OR IF ANXIETY DUE ONLY TO DELUSIONS → SECTION 4.

Cut off

HAVE YOU HAD TIMES WHEN YOU FELT SHAKY, OR YOUR HEART POUNDED, OR YOU FELT SWEATY, AND YOU SIMPLY HAD TO DO SOMETHING ABOUT IT?

कभी ऐसा हुआ है कि आप कंपने लगे हों, आपका दिल जोर से धड़कने लगा हो या सारे शरीर पर पसीना आ गया हो और जिस के लिये आपको कुछ न कुछ करना पड़ा हो?

(What is it like?)

(What was happening at the time?)

(How often during the past month?)

RATE PANIC ATTACKS WITH AUTONOMIC SYMPTOMS.

A panic attack is intolerable anxiety leading to some action to end it. e.g. leaving a bus, phoning husband at work, going in to see a neighbour, etc.

1 = One to four panic attacks during month

2 = Panic attacks five times or more.

DO YOU TEND TO GET ANXIOUS IN CERTAIN SITUATIONS, SUCH AS TRAVELLING, OR BEING ALONE, OR BEING IN A LIFT OR TUBE TRAIN?

क्या आप ऐसे मौकों पर जैसे बस या गाड़ी में सफर करने में, अकेले में, या तंग जगह पर जाने में घबरा जाते हैं?

(What situations? How often during the past month?)

(CHECK LIST : Can be presented on separate card and each item rated separately, if needed.

Crowds (shop, street, theatre, cinema, church).

Going out alone; being at home alone.

Enclosed spaces (hairdresser, phone booth, tunnel).

Open spaces, bridges.

Travelling (buses, cars, trains.)

RATE SITUATIONAL AUTONOMIC ANXIETY.

1 = Has not been in such situations during the past month but aware that anxiety would have been present if the situation had occurred.

2 = Situation has occurred during the past month and patient did feel anxious because of it.

WHAT ABOUT MEETING PEOPLE, e.g. GOING INTO A CROWDED ROOM, MAKING CONVERSATION?

लोगों से मिलने जुलने, बात करने या भरी सभा में या ऐसी जगह जाने में जहां बहुत भीड़ हो, आप कैसा महसूस करते हैं?

(CHECK LIST : Present card if necessary :

Speaking to an audience.

Eating, drinking or writing in front of other people, Parties.)

RATE AUTONOMIC ANXIETY ON MEETING PEOPLE

1 = Has not been in such situations during the past month but aware that anxiety would have been present if the situation had occurred.

2 = Situation has occurred during the past month and patient did feel anxious because of it.

DO YOU HAVE ANY SPECIAL FEARS, LIKE SOME PEOPLE ARE SCARED OF FEATHERS OR CATS OR SPIDERS OR BIRDS?

क्या आपको किसी खास चीज़ से डर लगता है, जैसे कुछ लोग कीड़े—मकोड़ों, बिल्ली, कुत्ता या छिपकली से डरते हैं?

(CHECK LIST : Present card if necessary : Heights, thunderstorms, darkness. Animals or insects of any kind. Dentists, injections, blood, injury.)

☐ (14)

☐ (15)

☐ (16)

RATE ONLY SPECIFIC PHOBIAS, NOT GENERAL SITUATIONAL ANXIETY.

- 1 = Has not been in such situations during the past month but aware that anxiety would have been present if the situation had occurred.
- 2 = Situation has occurred during the past month and patient did feel anxious because of it.

DO YOU AVOID ANY OF THESE SITUATIONS (SPECIFY AS APPROPRIATE) BECAUSE YOU KNOW YOU WILL GET ANXIOUS?

क्या आप इन अवसरों या मौकों से (जो मरीज़ पर लागू होते हैं) दूर रहने की कोशिश करते हैं क्योंकि इनसे आपको घबराहट होती है?

(How much does it affect your life?)

RATE AVOIDANCE OF ANXIETY-PROVOKING SITUATIONS.

- 1 = Subject tends to avoid such situations whenever possible.
- 2 = Marked generalisation of avoidance has occurred during past month, e.g. subject has not dared to leave the house or has gone out only if accompanied.

Describe anxiety symptoms and list phobias.

4. THINKING, CONCENTRATION, ETC.

**** CAN YOU THINK CLEARLY OR IS THERE ANY INTERFERENCE WITH YOUR THOUGHTS?**

क्या आप साफ साफ सोच सकते हैं या आपके सोच विचार में किसी तरह की रुकावट/बाधा/विघ्न आती है?

**** DO YOUR THOUGHTS TEND TO BE MUDDLED OR SLOW?**

क्या आपके विचारों में कोई गड़बड़ होती है जैसे लगता हो विचार उलझ गए या धीरे-धीरे आते हैं?

(Can you make up your mind about simple things quite easily?)
Make decisions about everyday matters?)

RATE SUBJECTIVELY INEFFICIENT THINKING (If due to intrusion of alien thoughts, rate 9).

- 1 = Symptom definitely present during the past month, but of moderate clinical intensity, or intense less than 50%
- 2 = Symptom clinically intense more than 50% of the past month

**** WHAT HAS YOUR CONCENTRATION BEEN LIKE RECENTLY?**

इन दिनों आपकी ध्यान लगाने की शक्ति कैसी रही है? अगर किसी काम में ध्यान लगाना हो तो क्या लग जाता है?

(Can you read an article in the paper or watch a TV programme right through?)

(Do your thoughts drift off so that you don't take things in?)

RATE POOR CONCENTRATION.

- 1 = Only moderate form of symptom present during the past month (e.g. can read a short article, can concentrate if tries hard); or intense less than 50% of the time.
- 2 = Symptom clinically intense (cannot attempt to read or concentrate) more than 50% of the past month.

**** DO YOU TEND TO BROOD ON THINGS?**

क्या आप अपने विचारों में उलझे/खोए रहते हैं जिससे काम में बाधा पड़ती है?

(So much that you even neglect your work?)

RATE NEGLECT DUE TO BROODING.

- 1 = Symptoms has caused moderate impairment to work or social relationships.

9

☐

(17)

☐

(18)

☐

(19)

☐

(20)

☐

(21)

**** WHAT ABOUT YOUR INTERESTS, HAVE THEY CHANGED AT ALL?**

आपके शौक कैसे हैं, क्या इनमें कोई फरक पड़ा है?

(Have you lost interest in work, or hobbies, or recreations?)

(Have you let your appearance go?)

RATE LOSS OF INTEREST *continuing during the past months.*

1 = Symptom definitely present during the past month, but of moderate clinical severity or severe loss less than 50% of the time.

2 = Symptom clinically severe more than 50% of the past month.

☐

(22)

**** HAVE YOU BECOME INTERESTED IN NEW THINGS AT ALL?**

क्या इन दिनों कोई नया शौक पैदा हुआ है?

IF EVIDENCE OF EXPANSIVE MOOD OR IDEAS → SECTION 9.

IF ODD IDEAS, EXPLORE FURTHER, PROCEED TO SECTION 15.

IF APPROPRIATE.

**** HAVE YOU SUFFERED ANY LAPSES OF MEMORY RECENTLY?**

क्या हाल में ऐसा हुआ है कि आप चीजें भूल जाते हों? ऐसा लगता है कि आप की याददाश्त कमजोर हो गई है?

IF EVIDENCE OF DISSOCIATION OR ORGANIC MEMORY LOSS →

SECTION 16.

ANSWERS TO THESE QUESTIONS MAY SUGGEST THAT OTHER TYPES OF THOUGHT DISORDER ARE PRESENT, IF NOT, CUT OFF →

SECTION 5.

Cut off

IF ANY EVIDENCE OF THOUGHT DISORDER :

ARE YOU IN FULL CONTROL OF YOUR THOUGHTS?

क्या आपके विचार आपके वश में हैं? या ऐसा लगता है कि विचार अपने वश में नहीं?

CAN PEOPLE READ YOUR MIND?

क्या लोग आपके मन की बात बिना बताये भी जान लेते हैं?

IS ANYTHING LIKE HYPNOTISM OR TELEPATHY

GOING ON?

क्या ऐसा लगता है कि कोई आप पर जादू-टोना कर रहा है?

IF NECESSARY, PROCEED TO SECTION 13.

5. DEPRESSED MOOD

**** DO YOU KEEP REASONABLY CHEERFUL OR HAVE YOU BEEN VERY DEPRESSED OR LOW-SPIRITED RECENTLY?**

आजकल आपका मन कैसा रहता है? आमतौर पर क्या आप खुश रहते हैं?

क्या आप इन दिनों उदास रहते हैं आपको कुछ भी अच्छा नहीं लगता है.

HAVE YOU CRIED AT ALL?

क्या आपको कभी रोना भी आया है?

(When did you last really enjoy doing anything?)

RATE DEPRESSED MOOD. N. B. *When rating clinical severity of depression remember that deeply depressed people may not necessarily cry. See definition in glossary.*

1 = Only moderately depressed during past month, or deep depression for less than 50% of the time and tending to vary in intensity.

2 = Deeply depressed for more than 50% of the past month, and tending to be unvarying in intensity.

☐

(23)

**** HOW DO YOU SEE THE FUTURE?**

आपको अपना भविष्य कैसा लगता है? आने वाले समय की तरफ देखें तो कैसा लगता है?

(Has life seemed quite hopeless?)

(Can you see any future?)

(Have you given up or does there still seem some reason for trying?)

RATE HOPELESSNESS *on subject's own view at present.*

(24)

1 = Hopelessness of moderate intensity but still has some degree of hope for the future (irrespective of time during month.)

2 = Intense form of symptom (patient has given up hope altogether).

USE JUDGEMENT ABOUT WORDING.

**** HAVE YOU FELT THAT LIFE WASN'T WORTH LIVING?**

कभी ऐसा भी लगता है कि यह जीवन बेकार है, इससे तो अच्छा है मर जाएं?

(Did you ever feel like ending it all?)

(What did you think you might do?)

(Did you actually try?)

RATE SUICIDAL PLANS OR ACTS.

(25)

1 = Deliberately considered suicide (not just a fleeting thought) but made no attempt.

2 = Suicidal attempt but subject's life never likely to be in serious danger, except unintentionally.

3 = Suicidal attempt apparently designed to end in death (i.e. accidental discovery or inefficient means).

N. B. Examiner should judge clinically whether there was intent to end life or not. If in doubt, assume not.

Cut off

IF EVIDENCE OF BOTH DEPRESSION AND ANXIETY RATE ANXIETY OR DEPRESSION PRIMARY.

If subject suffers from both anxiety and depression, and both have been rated as present, try to decide which is primary.

Which seems worse, the depression or the anxiety? (Use patient's own terms.)

आपको ज्यादा तकलीफ किस से रहती है — घबराहट से या उदासी से?

(26)

0 = Anxiety is primary. Depression appears to be entirely explicable in terms of the limitations placed on the subject by the symptoms of anxiety, e.g. being unable to leave the house, travel, meet people, etc., or being afraid of heart disease because of palpitations.

1 = Anxiety and depression both present but seem independent of each other or it is not possible to decide whether one of them is primary.

2 = Depression is primary. Anxiety is either a result of the depression (e.g. subject is frightened because of morbid or suicidal ideas) or it takes the form of fears of catastrophe, forebodings about illness or death, dread of having to face the day when first waking in the morning, preoccupation that something awful is going to happen. Panic attacks and situational anxiety, if present, are secondary to depression.

IS THE DEPRESSION WORSE AT ANY PARTICULAR TIME OF DAY?

☐ (27)

क्या किसी खास समय जैसे सुबह या शाम को उदासी ज्यादा होती है?

RATE MORNING DEPRESSION (*particularly on waking*).

0 = No depression.

1 = Not specially marked in mornings.

2 = Specially marked in mornings.

6. SELF AND OTHERS

** HAVE YOU WANTED TO STAY AWAY FROM OTHER PEOPLE?

क्या आप दूसरे लोगों से परे/दूर-अलग रहना पसन्द करते हैं?

(Why?)

(Have you been suspicious of their intentions? Of actual harm?)

☐ (28)

RATE SOCIAL WITHDRAWAL

1 = Only passive form of symptom, i.e. subject does not seek company but does not refuse it if offered; or, if active withdrawal, less than 50% of the month.

2 = Actively avoids company (refuses it if offered). Actively withdraws in this way for more than 50% of the month.

** WHAT IS YOUR OPINION OF YOURSELF COMPARED TO OTHER PEOPLE?

दूसरे लोगों के मुकाबले, आपकी अपने बारे में क्या राय है? क्या आप अपने को दूसरों से घटिया समझते हैं?

(Do you feel better, or not as good, or about the same as most?)

(Do you feel inferior or even worthless?)

☐ (29)

RATE SELF-DEPRECIATION.

1 = Some inferiority, not amounting to feeling of worthlessness. If subject considers self to be worthless, this intense form of the symptom is present less than 50% of the time.

2 = Subject considers self to be completely worthless. Symptom present more than 50% of the month.

** HOW CONFIDENT DO YOU FEEL IN YOURSELF?

आपको अपने आप पर कितना आत्मविश्वास/भरोसा लगता है?

(For example, in talking to others, or in managing your relations with other people?)

RATE LACK OF SELF-CONFIDENCE WITH OTHER PEOPLE.

Consider only competence in social relationships, not competence at mechanical work, etc.

☐ (30)

1 = Moderate lack of self-confidence, or intense lack less than 50% of the month.

2 = Intense lack of self-confidence, more than 50% of the month.

** ARE YOU SELF-CONSCIOUS IN PUBLIC?

क्या आपको लगता है कि लोग खासतौर से आपको ज्यादा देखते हैं?

(Do you get the feeling that other people are taking notice of you in the street or a bus or a restaurant?)

(Do they ever seem to laugh at you or talk about you critically?)

(Do you consider people really are looking at you, or is it perhaps the way you feel about it?)

RATE SIMPLE IDEAS OF REFERENCE (NOT DELUSIONS).

☐ (31)

1 = Marked self-consciousness only (irrespective of time during month.)

2 = Feels that people are criticising or laughing at self but can be reassured.

IF NO EVIDENCE OF GUILT, CUT OFF → SECTION 7.
(IF EVIDENCE OF MISINTERPRETATIONS, DELUSIONS OF
REFERENCE OR PERSECUTION → SECTIONS 15 B, 15 C.)

Cut off

IF EVIDENCE OF GUILT :

**DO YOU HAVE THE FEELING THAT YOU ARE
BEING BLAMED FOR SOMETHING, OR EVEN
ACCUSED? WHAT ABOUT?**

क्या ऐसा महसूस होता है कि आपको कसूरवार ठहराया जा रहा है, और यहां तक कि आप
पर इलजाम लगाये गये हैं - किन बातों के बारे में?

RATE GUILTY IDEAS OF REFERENCE. *Do not include justifiable
blame or accusation. Exclude delusions of guilt.*

- 1 = Subject feels blamed but not accused (irrespective of time during month).
2 = Subject feels accused of some sin or misdemeanour. Not delusional.

IF DELUSIONS OF REFERENCE MAY BE PRESENT → SECTION 15 B.

DO YOU TEND TO BLAME YOURSELF AT ALL?

क्या आप अपने आप को दोष देते हैं?

(If people are critical, do you think you deserve it?)

RATE PATHOLOGICAL GUILT ONLY.

- 1 = Subject feels over-guilty about some peccadillo (irrespective of time during month).
2 = Subject feels to blame for everything that has gone wrong even when not his fault, but not delusional.

IF DELUSIONS OF GUILT MAY BE PRESENT → SECTION 15 G.

DO YOU BLAME ANYONE ELSE FOR YOUR TROUBLES?

क्या आप अपनी तकलीफों के लिये किसी और को दोषी मानते हैं?

IF DELUSIONS OF PERSECUTION → SECTION 15 C

7. APPETITE, SLEEP, RETARDATION, LIBIDO

**** WHAT HAS YOUR APPETITE BEEN RECENTLY?**

इन दिनों आपकी भूख कैसी रही है?

(Have you lost any weight during the past three months?)

RATE LOSS OF WEIGHT DUE TO POOR APPETITE.

Do not include changes due to physical illness.

- 1 = Less than 7 lb (3.5 kg.)
2 = 7 lb (3.5 kg.) or more.

**** HAVE YOU HAD ANY TROUBLE GETTING OFF TO
SLEEP DURING THE PAST MONTH?**

क्या पिछले महीने आपको नींद आने में कोई तकलीफ रही?

(How long do you lie awake?)

(What happens if you take sleeping tablets?)

(How often does it happen?)

(32)

(33)

(34)

30)

(31)

(31)

RATE DELAYED SLEEP.

- 1 = One hour or more delay (irrespective of sleeping tablets).
 2 = Two hours or more delay (irrespective of sleeping tablets).
 (In either case, ten or more nights during month).

 (35)

**** DO YOU SEEM TO BE SLOWED DOWN IN YOUR MOVEMENTS, OR TO HAVE TOO LITTLE ENERGY RECENTLY? HOW MUCH HAS IT AFFECTED YOU?**

- ✓ क्या आपको ऐसे लगता है कि आप सुस्त हो गये हैं, कामकाज बड़ा धीरे धीरे करते हैं, जैसे आपमें ताकत कम हो? इसका आप पर कितना असर पड़ा है?

(Do things seem to be moving too fast for you?)

RATE SUBJECTIVE ANERGIA AND RETARDATION.

- 1 = Marked subjective listlessness and lack of energy.
 2 = Marked retardation and underactivity (Irrespective of time during month).

 (36)

IF NO APPETITE OR SLEEP DISTURBANCE, AND NO DEPRESSION,
 CUT OFF → SECTION 8.

Cut off

IF SLEEP DISTURBANCE OR DEPRESSION :

DO YOU WAKE EARLY IN THE MORNING?

✓ क्या आपकी नींद सुबह बहुत जल्दी खुल जाती है?

RATE EARLY WAKING (*one hour before usual*).

- 1 = One hour or more before ordinary time.
 2 = Two hours or more before ordinary time.
 (In either case, ten or more nights during month).

 (37)

HAS THERE BEEN ANY CHANGE IN YOUR INTEREST IN SEX?

✓ क्या आपकी सैक्स (आदमी औरत के सम्बन्ध) की इच्छा में कोई फर्क पड़ा है?

RATE LOSS OF LIBIDO WITHIN PRESENT EPISODE OF ILLNESS AND PERSISTING DURING PAST MONTH.

- 1 = Marked loss of interest and performance.
 2 = Almost total loss of libido.

 (38)

DOES THE DEPRESSION OR TENSION GET WORST JUST BEFORE THE START OF THE MONTHLY PERIOD?

✓ क्या माहवारी शुरू होने के पहले उदासी या मन में खिंचाव-तनाव बढ़ जाते हैं?

RATE PREMENSTRUAL EXACERBATION.

- 0 = No definite exacerbation.
 1 = Marked exacerbation.

 (39)
8. IRRITABILITY

**** HAVE YOU BEEN VERY MUCH MORE IRRITABLE THAN USUAL RECENTLY?**

✓ क्या आप इन दिनों पहले से बहुत ज्यादा चिड़चिड़े हो गये हैं?

(How do you show it?)

(Do you keep it to yourself, or shout, or even hit people?)

RATE IRRITABILITY.

- 1 = Keeps irritation to himself.
 2 = Shows anger by shouting or quarrelling.
 3 = Shows anger by hitting people, throwing or breaking things.

 (40)

9. EXPANSIVE MOOD AND IDEATION**** HAVE YOU SOMETIMES FELT PARTICULARLY CHEERFUL AND ON TOP OF THE WORLD, WITHOUT ANY REASON?**

✓ क्या आपको कभी बिना किसी खास वजह के बेहद खुशी महसूस हुई है?

(Too cheerful to be healthy?)

(How long does it last?)

RATE EXPANSIVE MOOD : *not ordinary high spirits.*

1 = Moderately expansive mood (euphoria with marked element of inappropriateness or excitement, whether recognised by subject or not), present during past month, and persistent for hours at a time.

* Do not include transient high spirits. Not necessarily described by subject.

2 = Intense form of symptom (elation or exaltation) definitely present during past month and persistent for hours at a time. Described by subject.

☐

(41)

**** HAVE YOU FELT PARTICULARLY FULL OF ENERGY LATELY, OR FULL OF EXCITING IDEAS?**

✓ क्या इन दिनों आपको ऐसा महसूस हुआ कि आप में बहुत ताकत या चुस्ती आ गई है? मैं नये नये विचार आने लगे हैं?

(Do things seem to go too slowly for you?)

(Do you need less sleep than usual?)

(Do you find yourself extremely active but not getting tired?)

(Have you developed new interests recently?)

RATE SUBJECTIVE IDEOMOTOR PRESSURE.

1 = Subjective equivalent of flight of ideas. Images and ideas flash through the mind, each suggesting others, at a faster rate than usual. State persists for hours at a time. * Definitely occurred during past months.

2 = As (1) but accompanied by very high energy output and activity which does not seem to make subject tired at the time. Definitely occurred during past month and persisted for hours at a time. *

☐

(42)

IF NO EVIDENCE OF EXPANSIVE MOOD AND IDEATION, CUT OFF SECTION 10.

Cut off

IF EVIDENCE OF EXPANSIVE MOOD AND IDEATION :

HAVE YOU SEEMED SUPER-EFFICIENT AT WORK, OR AS THOUGH YOU HAD SPECIAL POWERS OR TALENTS QUITE OUT OF THE ORDINARY?

✓ क्या लगता है कि आप अपने काम में बहुत माहिर हो गये हैं? आप में खास शक्ति या गुण आ गये हैं जो कि आम आदमी में नहीं होते?

HAVE YOU FELT SPECIALLY HEALTHY?

✓ क्या आपकी सेहत पहले से बहुत अच्छी हो गई है?

HAVE YOU BEEN BUYING ANY INTERESTING THINGS RECENTLY?

✓ क्या इन दिनों आप पहले से ज्यादा नयी नयी चीजें खरीदने लगे हैं? क्या आप पहले से ज्यादा पैसा खर्च करने लगे हैं?

RATE GRANDIOSE IDEAS AND ACTIONS.

1 = Subjective feeling of superb health, exceptionally high intelligence, extra ordinary abilities, etc. Persistent for hours at a time.*

Symptom occurred at some time during the month.

2 = Grandiose ideas have been translated into action during the month, e.g. overspending, gambling, etc. under the influence of grandiose ideas and expansive affect. Do not include compulsive gambling unless clearly of this type.

(→ GRANDIOSE DELUSIONS, SECTION 15D IF NECESSARY).

* If symptom was more transient but very intense or frequently repeated it may still be included.

☐

(43)

10. OBSESSIONS

These symptoms are usually experienced as occurring against conscious resistance (see definition in glossary).

**** DO YOU FIND THAT YOU HAVE TO KEEP ON CHECKING THINGS THAT YOU KNOW YOU HAVE ALREADY DONE?**

क्या ऐसे होता है कि आपको चीजों को बार बार जांच/चैक करना पड़ता है, यह जानते हुए भी कि आप पहले ऐसा कर चुके हैं?

(Like gas taps, doors, switches, etc.) (Do you have to touch or count things many times or repeat the same action over and over again?)
(What happens when you try to stop?)

RATE OBSESSIVE CHECKING AND REPEATING.

1 = Symptom of moderate intensity or, if severe, present less than 50% of the time.

2 = Symptom present in severe degree, more than 50% of the past month.

☐

(44)

**** DO YOU SPEND A LOT OF TIME ON PERSONAL CLEANLINESS LIKE WASHING OVER AND OVER EVEN THOUGH YOU KNOW YOU ARE CLEAN? WHAT ABOUT TIDINESS?**

क्या आप अपनी सफाई पर बहुत ज्यादा समय लगाते हैं, जैसे बार बार नहाना, हाथ धोना, यह जानते हुए भी कि आप साफ हैं? क्या चीजों को सजाने सवॉरने में भी बहुत समय लगाते हैं?

(Do you get worried by contamination with germs?)
(Do you have other rituals?)
(What happens when you try to stop?)

RATE OBSESSIVE CLEANLINESS AND SIMILAR RITUALS.

1 = Symptom of moderate intensity or, if severe, present less than 50% of the time.

2 = Symptom present in severe degree, more than 50% of the past month.

☐

(45)

**** DO YOU FIND IT DIFFICULT TO MAKE DECISIONS EVEN ABOUT TRIVIAL THINGS?**

क्या छोटी छोटी बातों का फैसला करने में भी बहुत मुश्किल होती है?

(Do you constantly have to question the meaning of the universe?)

(Do you get awful thoughts coming into your mind even when you try to keep them out?)
(What happens when you try to stop?)

RATE OBSESSIVE IDEAS AND RUMINATION.

1 = Symptom of moderate intensity or, if severe, present less than 50% of the time.

2 = Symptom present in severe degree, more than 50% of the past month.

☐

(46)

11. DEREALISATION AND DEPERSONALISATION.

**** HAVE YOU HAD THE FEELING RECENTLY THAT THINGS AROUND YOU WERE UNREAL?**

क्या इन दिनों आपको अपने आस पास चीजें बदली बदली महसूस होती हैं?

(As though everything was an imitation of reality, like a stage set, with people acting instead of being themselves?)

(What is it like? How do you explain it?)

RATE DEREALISATION.

1 = Moderately intense form of symptom definitely occurred during the past month, and persisted for hours at a time. Things appear colourless and artificial, people appear lifeless and seem to act rather than being themselves.

☐

(47)

- 2 = Intense form of symptom occurred during the past month and persisted for hours at a time, e.g. whole world appears like a gigantic stage set, with imitation instead of real objects and puppets instead of people. (If delusional, do not rate here but symptom 90.)

**** HAVE YOU YOURSELF FELT UNREAL, THAT YOU WERE NOT A PERSON, NOT IN THE LIVING WORLD?**

✓ क्या आप खुद भी बदले बदले महसूस करते हैं जैसे आप वास्तव में इन्सान नहीं है या इस दुनिया में नहीं हैं?

(Or that you were outside yourself, looking at yourself from outside?)

(Or that you look unreal in the mirror?)

(Or that some part of your body did not belong to you?)

(How do you explain it?)

RATE DEPERSONALISATION

- 1 = Moderately intense form of the symptom definitely occurred during the past month and persisted for hours at a time. Subject feels himself unreal, a sham, a shadow.
- 2 = Intense form of symptom definitely occurred during the past month and persisted for hours at a time. Subject feels he is dead, not a person, living in a parallel existence, a hollow shell, even that he does not exit. (If delusional, do not rate here but symptom 90.)

☐ (48)

12. OTHER PERCEPTUAL DISORDERS (NOT HALLUCINATIONS)

**** DO YOU EVER GET THE FEELING THAT SOMETHING ODD IS GOING ON WHICH YOU CAN'T EXPLAIN?**

✓ क्या आपको ऐसा लगता है जैसे कोई चीज़ अजीब सी हो रही है जिसे आप ठीक से समझ नहीं पा रहे हैं?

(Or that familiar surroundings seem strange? How do you explain it?)

RATE DELUSIONAL MOOD : *The subject feels that his familiar environment has changed in a way which puzzles him and which he may not be able to describe clearly. The feeling often accompanies delusion formation.*

- 1 = Symptom definitely present. No delusions have actually been formulated, though patient may feel that various delusional explanations are possible.

- 2 = Full delusional elaboration has occurred.

☐ (49)

**** DOES YOUR IMAGINATION SOMETIMES PLAY TRICKS ON YOU?**

✓ क्या आपकी कल्पना आपको धोखा (सुनने-देखने में धोखा) भी दे जाती है?

**** IS THERE ANYTHING UNUSUAL ABOUT THE WAY THINGS LOOK OR SOUND, OR SMELL, OR TASTE?**

✓ क्या ऐसे लगता है कि चीज़ों की शक्ति, आवाज़, गन्ध या स्वाद अजीब है, वैसे नहीं जैसे कि होना चाहिये?

(Does your body function normally?)

(Is your own appearance normal?)

CONTINUE BELOW CUT-OFF IF NECESSARY, EVEN IF (49) NOT PRESENT.

IF NO PERCEPTUAL ABNORMALITY—→ SYMPTOM 54.

Cut off

IF THERE IS ANY HINT OF PERCEPTUAL ABNORMALITY, CONTINUE BEYOND CUT-OFF AND ALSO CONSIDER LATER SECTIONS.

RATE ONLY BASIC EXPERIENCE. NOT DELUSIONAL ELABORATION.

**** IN WHAT WAY? DO SOUNDS SEEM UNNATURALLY CLEAR OR LOUD, OR THINGS LOOK VIVIDLY COLOURED OR DETAILED?**

क्या आवाज़ें कुछ ज्यादा ही साफ और ऊंची महसूस होने लगी हैं? हर चीज़ बड़ी रंगीन और साफ साफ दिखाई देने लगी हैं?

(How do you explain this?)

RATE HEIGHTENED PERCEPTION : e.g. *subject intensely aware of cracks in a wall, details of a wall paper pattern, colours in a picture. Sounds heard with exceptional clarity, music appears particularly beautiful.*

- 1 = Subject unable to describe the symptom precisely, but examiner thinks it is likely to have been present at some time during the past month.
- 2 = Subject describes symptom. Definitely present at some time (even if only briefly) during the past month.

**** DO THINGS SEEM DARK OR GREY OR COLOURLESS?**

क्या चीज़ें काली-सी, धुंधली-सी या बेरंगी सी महसूस होने लगी हैं?

RATE DULLED PERCEPTION : *The reverse of symptom (50). Things look, sound and taste dull, flat, colourless and uninteresting.*

- 1 = Subject unable to describe the symptom precisely, but examiner thinks it is likely to have been present at some time during the past month.
- 2 = Subject describes symptom. Definitely present at some time (even if only briefly) during the past month.

**** DOES THE APPEARANCE OF THINGS OR PEOPLE CHANGE IN A PUZZLING WAY : e. g. DISTORTED SHAPES OR SIZE OR COLOUR?**

क्या लोग या चीज़ें कुछ अजीब ढंग से बदली - बदली नज़र आती हैं - जैसे उनकी शक्ल बदल गई हो या छोटी - बड़ी लगती हों या रंग में फर्क लगता हो?

(How do you explain it?)

RATE CHANGED PERCEPTION.

- 1 = Subject unable to describe the symptom precisely, but examiner thinks it is likely to have been present at some time during the past month.
- 2 = Subject describes symptom. Definitely present at some time (even if only briefly) during the past month.

**** DO YOU THINK YOUR OWN APPEARANCE IS NORMAL?**

क्या ऐसा लगता है कि इन दिनों आपकी शक्ल - सूरत में कोई फर्क आ गया है?

(Conviction that nose is too large, teeth, misshapen, body crooked, etc. Ask questions here if convenient but rate symptom (89).)

**** DOES YOUR EXPERIENCE OF TIME SEEM TO HAVE CHANGED?**

क्या आपको ऐसे लगता है कि समय बहुत तेज़ी से या बहुत धीरे धीरे बीत रहा है?

(Does it go too fast too slowly, or do you seem to live through experiences exactly as you have had them before?)

☐ (50)

☐ (51)

☐ (52)

RATE CHANGED PERCEPTION OF TIME, INCLUDING DEJA VU.

- 1 = Subject unable to describes the symptom precisely, but examiner thinks it is likely to have been present at some time during the past month.
- 2 = Subject describe symptom. Definitely present at some time (even if only briefly) during the past month.

☐ (53)

DO YOU FEEL YOU HAVE LOST YOUR EMOTIONS IN SOME WAY?

✓ क्या ऐसा है कि खुशी-गमी वगैरह कुछ भी महसूस नहीं होती?

(That you are empty of all feeling, incapable of reacting emotionally?)
(Is this a definite change, or have you always been like that?)
(How do you explain it?)

0)

RATE LOST EMOTIONS : *Rate only subjective loss of affect, i.e. subject can remember being able to react emotionally, though this might have been months or even years ago.*

- 1 = Symptom definitely present during the past month but less than 50% of the time.
- 2 = Symptom present more than 50% during the past month.

☐ (54)

13. THOUGHT READING, INSERTION, ECHO, BROADCAST
IF QUESTION HAS NOT BEEN COVERED IN SECTION 4 ASK :

**** CAN YOU THINK QUITE CLEARLY OR IS THERE ANY INTERFERENCE WITH YOUR THOUGHTS?**

✓ क्या आप साफ साफ सोच सकते हैं या आपके सोच विचार में किसी तरह की रुकावट/बाधा/विघ्न आती है?

1)

(Are you in full control of your thoughts?)
(Can people read your mind?)
(Is anything like hypnotism or telepathy going on?)

IF NO EVIDENCE OF THOUGHT READING, etc. CUT OFF → SECTION 14.

Cut off

IF ANY EVIDENCE, ASK QUESTIONS BELOW.

(These symptoms are often recorded as false positives. The examiner must be satisfied that the subject is not simply assenting to a question he does not understand, but genuinely recognises the experience and can describe it so that the examiner recognises it.) It is particularly important to know the relevant sections of the Instruction Manual well before rating these symptoms.

52)

52)

ARE THOUGHTS PUT INTO YOUR HEAD WHICH YOU KNOW ARE NOT YOUR OWN?

✓ क्या आपको लगता है कि जबरदस्ती आपके मन में विचार डाले जाते हैं? और आपको पूरा विश्वास है कि विचार आपके अपने नहीं हैं?

(How do you know they are not your own?)
(Where do they come from?)

RATE THOUGHT INSERTION : *Include only thoughts recognised as alien. Do not include delusional elaboration, only basic experience. (Exclude hallucinations.)*

- 1 = Symptom described clearly, but subject thinks it may be due to 'own unconscious thoughts' etc., i.e. not certainly alien.
- 2 = Symptom described clearly and thoughts described as alien, i.e. inserted into mind from elsewhere (even if subject does not know from where). Not hallucinations.

☐ (55)

**DO YOU EVER SEEM TO HEAR YOUR OWN THOUGHTS
SPOKEN ALOUD IN YOUR HEAD, SO THAT SOMEONE
STANDING NEAR MIGHT BE ABLE TO HEAR THEM?**

✓ जो विचार आपके मन में आते हैं, क्या आप उन्हें सुन भी सकते हैं? इतनी ऊंची आवाज़ में कि आसपास खड़े दूसरे लोग भी उन्हें सुन सकते हों?

(Are your thoughts broadcast, so that other people know what you are thinking?)

(How do you explain?)

RATE THOUGHT BROADCAST.

- 1 = Hears own thoughts 'spoken' aloud but not broadcast. Subject must really hear them aloud in his head. If in doubt rate (8) or (0).
- 2 = Thoughts transferred or broadcast so that others can share subject's thoughts even when they are not in the same room (Do not include 'thoughts being read' unless this is an explanation of thought broadcast. The subject must actually experience his thoughts being available to others).

**DO YOU EVER SEEM TO HEAR YOUR OWN THOUGHTS
REPEATED OR ECHOED ?**

✓ कभी ऐसा लगता है कि जो भी आप सोचते हैं वही विचार आवाज़ के रूप में दोहराया जाता है या गूँजता है?

(What is that like? How do you explain it?)

(Where does it come from?)

RATE THOUGHT ECHO OR COMMENTARY.

- 1 = Thought echo. If any doubt, rate (8) or (10).
- 2 = Subject experiences alien thoughts related to his own thoughts, i.e. associations or comments on his own thoughts. Not hallucinations.

☐

(57)

**DO YOU EVER EXPERIENCE YOUR THOUGHTS
STOPPING QUITE UNEXPECTEDLY SO THAT THERE
ARE NONE LEFT IN YOUR MIND, EVEN WHEN YOUR
THOUGHTS WERE FLOWING FREELY BEFORE?**

क्या कभी ऐसे हुआ कि एकदम आपके मन में विचार आने बन्द हो गये और आपका दिमाग बिल्कुल खाली हो गया जबकि इससे पहले विचार ठीक चल रहे थे?

(What is that like?)

(How often does it occur? What is it due to?)

**DO YOUR THOUGHTS EVER SEEM TO BE TAKEN OUT
OF YOUR HEAD, AS THOUGH SOME EXTERNAL PERSON
OR FORCE WERE REMOVING THEM?**

कभी ऐसे लगता है कि आपके दिमाग से विचार निकाल लिये गये हैं जैसे कोई बाहरी ताकत या कोई आदमी आपके विचार निकाल लेता हो?

(Can you give an example?)

(How do you explain it?)

RATE THOUGHT BLOCK OR WITHDRAWAL.

- 1 = Thought block. Do not include if due to anxiety or lack of concentration; only if it occurs totally unexpectedly when thoughts are flowing freely. One single occasion is not sufficient for rating. *Be very critical in rating this symptom.*
- 2 = Delusional explanation that thoughts are withdrawn.

☐

(58)

CAN ANYONE READ YOUR THOUGHTS?

क्या बिना बताये कोई आपके मन की बात जान लेता है?

(How do you know?)

(How do you explain it?)

RATE DELUSION OF THOUGHTS BEING READ :

only if subject does not mean that people can infer his thoughts from his actions. (Do not include subject reading thoughts of other people → 76.)

☐

(59)

- 1 = 'Partial' delusion. Subject entertains the possibility that thoughts might be read but is not certain about it. Exclude if subcultural explanation.
- 2 = Full delusion. Exclude if subcultural explanation. The term 'thought reading' is commonly used to mean the ability to tell what someone is thinking from the way they behave—this use should be excluded.

14. HALLUCINATIONS

USE JUDGEMENT ABOUT WORDING.

- ** I SHOULD LIKE TO ASK YOU A ROUTINE QUESTION WHICH WE ASK OF EVERYBODY. DO YOU EVER SEEM TO HEAR NOISES OR VOICES WHEN THERE IS NO ONE ABOUT, AND NOTHING ELSE TO EXPLAIN IT?**

✓ अब मैं आपसे एक ऐसा सवाल पूछूंगा जो अक्सर हम सब से पूछते हैं — क्या ऐसा होता है जब आप अकेले होते हैं, और आसपास कोई नहीं होता तब कानों में शोर या आवाजें सुनाई पड़ती हैं? कभी कभी ऐसा लगता है कि कोई आप का नाम पुकार रहा है?
(Do you ever seem to hear your name being called?)

- ** IS THAT TRUE OF VISIONS OR OTHER UNUSUAL EXPERIENCES, WHICH SOME PEOPLE HAVE? (TOUCH, TASTE, SMELL, TEMPERATURE, PAIN, etc.)**

✓ क्या इसी तरह से कुछ चीजें या शक्तें भी दिखाई देती हैं? जब आसपास कोई नहीं हो तो भी? इस तरह की कोई और अजीब बात हुई हो तो बतायें?

IF NO EVIDENCE FOR HALLUCINATIONS OF ANY SENSE,
CUT-OFF → SECTION 15.

Cut off

IF EVIDENCE FOR NON-AUDITORY HALLUCINATIONS ONLY →
SUBSECTIONS 14B and 14C.

14 A. AUDITORY HALLUCINATIONS.

IF ANY EVIDENCE THAT AUDITORY HALLUCINATIONS MIGHT BE PRESENT :

DO YOU HEAR NOISES LIKE TAPPING, OR MUSIC?

✓ क्या टक-टक या गाने की कोई आवाज सी सुनाई देती है?
(WHAT IS IT LIKE?)

DOES IT SOUND LIKE MUTTERING OR WHISPERING?

✓ यह आवाज कैसी होती है - खसर पुसर की तरह या जैसे कोई धीरे धीरे मुंह में बोल रहा हो?

CAN YOU MAKE OUT THE WORDS?

✓ क्या लफ्ज (शब्द) साफ साफ सुनाई देते हैं ?

RATE NON-VERBAL AUDITORY HALLUCINATIONS.

- 1 = Music, tapping, car engines, etc. Do not include tinnitus.
2 = Muttering, whispering but subject cannot make out any words at all.

WHAT DOES THE VOICE SAY?

✓ आवाज क्या कहती है?

(Write down examples of typical verbal hallucinations.)

(If accusatory : Do you think that it is justified? Do you deserve it?)

DO YOU HEAR YOUR NAME BEING CALLED?

✓ क्या आपका नाम भी पुकारा जाता है?

RATE VERBAL HALLUCINATIONS BASED ON DEPRESSION OR ELATION OR VOICE CALLING SUBJECT.

Content is congruent with mood; e.g. 'He's dirty', in context of depression, or 'Go to Westminster', in elated subject who thinks he is Prime Minister. Include voice calling subject (e.g. calling name) or saying single words only. Be careful to distinguish from delusions of reference in which people whom the subject can see are thought to be talking about him.

(60)

(61)

RECORD EXAMPLES

- 1 = Voice calling name, or single words only.
- 2 = Other verbal hallucinations, congruent with depressed mood.
- 3 = Other verbal hallucinations; congruent with elated mood.

DO YOU HEAR SEVERAL VOICES TALKING ABOUT YOU?

✓ क्या ऐसे लगता है कि कई आवाजें आपके बारे में बातें करती हैं?

DO THEY REFER TO YOU AS 'HE' (SHE)?

✓ क्या ये आवाजें 'वह' कर कर बुलाती हैं, जैसे 'वह ऐसे हैं', 'वह यह कर रहा है'?

(What do they say?)

(Do they seem to comment on what you are thinking, or reading, or doing?)

RATE VOICE(S) DISCUSSING SUBJECT IN THIRD PERSON OR COMMENTING ON THOUGHTS OR ACTIONS (NOT BASED ON DEPRESSION OR ELATION).

Do not include muttering or whispering if subject cannot make out words. Exclude 'dissociative' hallucinations (symptom 64). Do not include voice calling name or affectively based verbal hallucinations (symptom 61).

There may be one voice commenting on subject's thoughts or actions, or several voices discussing the subject in the third person.

RECORD EXAMPLES

- 1 = Hears a voice or voices commenting on thoughts or actions in third person (e.g. 'Now he's going to go to bed' or 'Why would he think a thing like that?'). (2) not present.
- 2 = Hears voices talking about him/her in third person (e.g. 'I think he's a homosexual, don't you?' 'Yes, he wears a pink pullover, that's a sign of it.'). (1) may also be present.

DO THEY SPEAK DIRECTLY TO YOU?

✓ क्या ये आवाजें सीधे आपसे बातचीत करती हैं?

(Are they threatening or unpleasant?)

(Do they call you names?)

DO THEY GIVE ORDERS? (DO YOU OBEY?)

✓ क्या ये आवाजें आपको हुकम (आदेश) भी देती हैं? क्या आप वैसा करते हैं?

RATE VOICE(S) SPEAKING TO SUBJECT (NOT BASED ON DEPRESSION OR ELATION).

Include voice(s) speaking directly to subject, whether accusing, threatening, giving orders or giving information. Exclude voice(s) calling name or based on depression or elation (symptom 61), or commenting on subject's thoughts or actions (symptom 62). Exclude 'dissociative' hallucinations (symptom 63.)

RECORD EXAMPLES.

- 1 = Pleasant, supportive or neutral voice(s), not based on affect. No hostile voices.
- 2 = Hostile, threatening or accusing voice(s), thought to be undeserved and not based on affect.

N. B. If single isolated words, even with neutral affect, include under 61(1).

CAN YOU CARRY ON A TWO-WAY CONVERSATION WITH — ?

✓ क्या आप भी उनसे बातचीत कर सकते हैं?

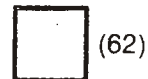
(You can reply, and then—replies to you, and you reply again, just as in an ordinary conversation?)

(Do you see anything, or smell anything, at the same time as you hear the voice?)

(Who is it you are talking to?)

(What is the explanation?)

(Do you know anyone else who has this kind of experience?)



RATE 'DISSOCIATIVE' HALLUCINATIONS (VERBAL AND/OR OTHER)

The subject can hold a two-way conversation with a presence (variously described as a person, ghost, spirit, god, etc.) which may also be sensed in other ways, e.g. visually or by touch or smell. Often connected with people with whom the subject has had strong affective ties. Visual hallucinations can occur alone. There is usually a strong subcultural colouring, e.g. the subject belongs to a religious sect or to a subcultural group which sanctions hallucinatory experiences, or the subject has been under the influence of someone who is involved with such practices. Exclude hypnagogic hallucinations.

RECORD EXAMPLES.

- 1 = 'Dissociative' hallucinations present. Subject belongs to subcultural group or sect in which such experiences are sanctioned.
2 = 'Dissociative' hallucinations present. Subject does not belong to sub-cultural group as in (1). If not known, rate (1).

ARE THESE VOICES IN YOUR MIND OR CAN YOU HEAR THEM THROUGH YOUR EARS?

✓ क्या ये आवाजें आपके मन से उठती हैं या बाहर से आती हैं और कानों से सुनाई देती हैं?
scoring:—

- 1 = Subject hears both pseudo-hallucinations (within mind) and true hallucinations (through ears).
2 = Subject hears pseudo-hallucinations only.
3 = Subject hears true hallucination only.

HOW DO YOU EXPLAIN THE VOICE?

✓ क्या ये आवाजें कहाँ से आती हैं, क्यों आती हैं, कैसे आती हैं, क्या इसके बारे में कुछ बता सकते हैं?

RECORD EXPLANATION.

14B. VISUAL HALLUCINATIONS

IF QUESTION HAS NOT BEEN COVERED IN SECTION 12 OR 14A, ASK:

** HAVE YOU HAD VISIONS, OR SEEN THINGS OTHER PEOPLE COULDN'T SEE?

✓ क्या आपको ऐसी चीजें दिखाई देती हैं जिन्हें दूसरे लोग नहीं देख सकते, केवल (सिर्फ) आप ही देख सकते हैं?

IF NO EVIDENCE, HERE OR ELSEWHERE, FOR VISUAL HALLUCINATIONS CUT OFF → SECTION 15.

Cut off	
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IF ANY EVIDENCE OF VISUAL HALLUCINATIONS:

WITH YOUR EYES OR IN YOUR MIND?

✓ क्या ये चीजें आपके मन में होती हैं या आंखों से साफ साफ दिखाई देती हैं?
WHAT DID YOU SEE? आपने क्या देखा था?

WERE YOU HALF ASLEEP AT THE TIME? क्या उस समय आप थोड़ी नींद में थे?

HAS IT OCCURRED WHEN YOU WERE FULLY AWAKE?

✓ जब आप पूरे जाग रहे हों, क्या ऐसा कभी हुआ?

DID YOU REALISE YOU WERE 'SEEING THINGS'?

✓ उस समय पता था कि यह ओपरी या बाहरी चीज दीख रही है?

DID THE VISION SEEM TO ARISE OUT OF A PATTERN ON THE WALLPAPER OR A SHADOW?

✓ क्या ऐसे लगा कि जो आप देख रहे हैं वह किसी परछाई या तस्वीर से निकल रहा है?
HOW DO YOU EXPLAIN IT? यह सब कैसे होता है, बता सकते हैं?

RATE VISUAL HALLUCINATIONS : *in clear consciousness including pseudo-hallucinations. Exclude 'dissociative' visual hallucinations (symptom 64).*

☐ (66)

- 1 = Formless visual hallucinations – flashes of light, shadows, etc.
2 = Formed visual hallucinations – people, objects like a 'fiery cross', faces, etc.

RATE DELIRIOUS VISUAL HALLUCINATIONS.

☐ (67)

14C. OTHER HALLUCINATIONS.

IF QUESTION HAS NOT BEEN COVERED IN PREVIOUS SECTIONS.

** IS THERE ANYTHING UNUSUAL ABOUT THE WAY THINGS FEEL, OR TASTE, OR SMELL?

क्या चीजों की शकल, स्वाद और गन्ध में कोई अजीब सी बात लगती है, कोई फर्क लगता है?

** DOES YOUR BODY FUNCTION NORMALLY?

क्या आपका शरीर ठीक-ठाक काम करता है या इसमें कोई फर्क लगता है?

IF NO EVIDENCE FOR OTHER HALLUCINATIONS CUT-OFF → SECTION 15A.

Cut off

IF ANY EVIDENCE FOR OTHER HALLUCINATIONS.

DO YOU SOMETIMES NOTICE STRANGE SMELLS THAT OTHER PEOPLE DON'T NOTICE?

✓ क्या आपको अजीब सी बू महसूस होती है जो औरों को नहीं होती?

(What sort of things ?)

(How do you explain it?)

RATE OLFACTORY HALLUCINATION : *Exclude delusion that patient himself smells.*

☐ (68)

- 1 = Simple olfactory hallucination. Not delusionally elaborated. Subject smells oranges, death, a burn smell, scent, etc., which other people cannot smell. Can offer no explanation.

- 2 = Delusional elaboration in addition, e.g. gas being put into room.

DO YOU SEEM TO THINK THAT YOU YOURSELF GIVE OFF A SMELL WHICH IS NOTICED?

✓ क्या ऐसा लगता है कि आप से बदबू आती है, जिसका दूसरों को भी पता चल जाता है?

(What is the explanation?)

RATE DELUSION THAT SUBJECT SMELLS : *Do not include simple preoccupation with body odour e.g. in anxious subject who sweats a lot.*

☐ (69)

- 1 = Subject irrationally thinks he gives off a smell but is not certain. Not sure that other have noticed it but thinks it possible.
2 = Subject sure that he gives off a smell and that others have noticed it and react accordingly.

DO YOU EVER FEEL THAT SOMEONE IS TOUCHING YOU BUT WHEN YOU LOOK THERE IS NOBODY THERE?

✓ कभी ऐसे लगता है जैसे किसी ने आपको छुआ हो, लेकिन आसपास देखने पर कोई नहीं होता?

(Have you noticed that food or drink seems to have an unusual taste recently?)

3)

RATE OTHER HALLUCINATIONS AND DELUSIONAL ELABORATION :
Exclude hypochondriacal and nihilistic delusions rated in (90) and (91).



(70)

- 1 = Sensation of touch, food tastes burnt, etc., but subject puzzled by the experience. No delusional elaboration.
- 2 = Delusional elaboration in addition, e.g. fantasy lover, food poisoned, etc.

)

3)

3)

3)

39)

15. DELUSIONS

Definition

Delusions may be of two kinds, primary and secondary. Both kinds are rated together in the following symptoms except where specified. For example, primary delusions are specifically rated in symptom (82). They are defined here for convenience.

Primary delusions are based upon experiences in which a subject suddenly becomes convinced that a particular set of events has a special meaning (e.g. a subject undergoing a liver biopsy suddenly felt he had been chosen by God.) This delusion cannot be explained and it is not shared by other members of the subject's cultural or social group.

Secondary delusions are delusional elaborations of primary delusions or other basic phenomena such as derealisation, depersonalisation, perceptual distortions, hallucinations, thought echo, mood changes, etc.

Above cut-off questions, likely to elicit delusions if present, are included in many of the preceding sections. There may also be evidence in the case-record or in the subject's spontaneous account.

IF NO EVIDENCE AT ALL THAT DELUSIONS ARE PRESENT, CUT OFF → SECTION 16.

RECORD IF ANY PSYCHOTIC PHENOMENA PRESENT, OTHER THAN DELUSIONS, USE JUDGEMENT AS TO WHETHER TO PROCEED BEYOND CUT-OFF.

Cut off	
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IF ANY EVIDENCE FOR DELUSIONS, ASK ALL QUESTIONS NOT IN BRACKETS, AND ANY FURTHER QUESTIONS WHICH SEEM INDICATED.

RATING OF PARTIAL AND FULL DELUSIONS.

In general, all delusions are rated as follows :

- 1 = Partial delusions, which are expressed with doubt, or as possibilities which the subject entertains but is not certain about. This rating should not be used if it is clear that full delusions have been present during the month, or if the subject has acted as if fully deluded.
- 2 = Full delusions have been present at some time during the month. Fully convinced. No insight.

A useful question to elucidate the differences between partial and full delusions is as follows :

EVEN WHEN YOU SEEM TO BE MOST CONVINCED, DO YOU REALLY FEEL IN THE BACK OF YOUR MIND THAT IT MIGHT WELL NOT BE TRUE, THAT IT MIGHT BE IMAGINATION?

✓ इसके बारे में चाहे आपको पूरा यकीन हो फिर भी कभी ऐसा लगता है कि शायद यह सब नहीं है या मन का वहम है?

15A. DELUSIONS OF CONTROL

Definition

The subject's will is replaced by that of some external agency. A simple statement that the radio is controlling the subject is not sufficient. (This statement, alone, should be rated 8.) The subject must describe a replacement of will by some other force.

Do not include feeling that life is planned and directed by fate, or that the future is present already in embryo, or that subject is not very strong-willed, or that voices give subject orders. Do not include simple identification with God or being under God's direction. Do not include subcultural or hysterical possession states of multiple personality. (→ 100).

DO YOU FEEL UNDER THE CONTROL OF SOME FORCE OR POWER OTHER THAN YOURSELF?

✓ क्या ऐसा महसूस होता है कि आपको किसी बाहर की ताकत या शक्ति ने अपने बश में कर रखा है?

(As though you were a robot or a zombie without a will of your own?)

(As though you were possessed by someone or something else?)

(What is that like?)

(Does this force make your movements for you without your willing it, or use your voice, or your handwriting? Does it replace your personality? What is the explanation?)

RATE DELUSIONS OF CONTROL.

- 1 = Partial delusions
- 2 = Full delusions.

15B. MISINTERPRETATIONS, MISIDENTIFICATION AND DELUSIONS OF REFERENCE

Definition

Delusions of reference : Do not include simple self-consciousness or feeling that subject attracts comment, even if critical. These are rated under symptom 31.



(71)

There must be elaboration : e.g. someone crosses his knees in order to indicate that the subject is homosexual; or the whole neighbourhood is gossiping.

Delusional misinterpretations, etc. This is an extension of the delusion of reference so that not only do people seem to refer to subject, but situations appear to be deliberately created to test him (exclude situations of medical treatment), or objects appear to have special meanings.

DO PEOPLE SEEM TO DROP HINTS ABOUT YOU OR SAY THINGS WITH A DOUBLE MEANING, OR DO THINGS IN A SPECIAL WAY SO AS TO CONVEY A MEANING?

✓ क्या ऐसा लगता है कि लोग आपकी ओर इशारा करके बातें करते हैं या ऐसे लगता है कि लोगों की बातों के दोहरे मतलब होते हैं या लोग ऐसे काम करते हैं जिनका आपके लिये खास मतलब होता है?

DOES EVERYONE SEEM TO GOSSIP ABOUT YOU?

✓ क्या ऐसे लगता है कि सभी लोग आपके बारे में बातें करते रहते हैं?

(Do people follow you about or check up on you or record your movements?)

(How do they do it? Why?)

(Are there people about who are not what they seem to be?)

RATE DELUSIONS OF REFERENCE.

1 = Partial delusions

2 = Full delusions.

DO THINGS SEEM TO BE SPECIALLY ARRANGED?

✓ क्या ऐसा लगता है कि बहुत सी बातें किसी खास ढंग से कही या की जा रही हैं?

(Is an experiment going on, to test you out?)

(Do you see any reference to yourself on TV or in the papers?)

(Do you ever seem to see special meanings in advertisements, or shop window, or in the way things are arranged?)

(How do you explain this?)

RATE DELUSIONAL MISINTERPRETATION AND MISIDENTIFICATION

1 = Partial delusions

2 = Full delusions

15C. DELUSIONS OF PERSECUTION

IS ANYONE DELIBERATELY TRYING TO HARM YOU, e.g. TRYING TO POISON YOU OR KILL YOU?

✓ क्या कोई आपको जानबूझकर नुकसान पहुंचाने की कोशिश कर रहा है? जैसे आपको जहर देने या मार देने की कोशिश कर रहा हो ?

(How? Is there an organisation like the Mafia behind it?)

(Is there any other kind of persecution? How do you explain this?)

RATE DELUSIONS OF PERSECUTION

1 = Partial delusions

2 = Full delusions

15D. EXPANSIVE DELUSION

DO YOU THINK THAT PEOPLE ARE ORGANISING THINGS SPECIALLY TO HELP YOU?

✓ क्या ऐसे लगता है कि लोग आपकी मदद करने के लिये खास तौर से इन्तजाम कर रहे हैं?

RATE DELUSIONS OF ASSISTANCE

1 = Partial delusions

2 = Full delusions.

IS THERE ANYTHING SPECIAL ABOUT YOU? DO YOU HAVE SPECIAL ABILITIES OR POWERS?

✓ क्या आपमें कोई खास बात है? आपमें कोई खास ताकत या शक्ति आ गई है?

(Can you read people's thoughts?)

(Is there a special purpose or mission to your life?)

(Are you specially clever or inventive? How do you explain this?)

☐ (72)

☐ (73)

☐ (74)

☐ (75)

RATE DELUSIONS OF GRANDIOSE ABILITIES.

- 1 = Partial delusions
2 = Full delusions

(Are you a very prominent person or related to someone prominent, like Royalty?)

(Are you very rich or famous?)

(How do you explain this?)

RATE DELUSIONS OF GRANDIOSE IDENTITY : (Exclude religious identification).

- 1 = Partial delusions
2 = Full delusions

5E DELUSIONS CONCERNING VARIOUS TYPES OF INFLUENCE AND PRIMARY DELUSIONS.

ARE YOU A VERY RELIGIOUS PERSON?

✓ क्या आप बहुत धार्मिक विचारों के हैं?

(Specially close to Christ or God?)

(Can God communicate with you? How?)

(Are you yourself a saint?)

(How do you explain this?)

RATE RELIGIOUS DELUSIONS : Including delusional religious explanations of other experiences. Exclude intense religious belief or purely subcultural beliefs.

- 1 = Partial delusions
2 = Full delusions

HOW DO YOU EXPLAIN THE THINGS THAT HAVE BEEN HAPPENING? (SPECIFY)

✓ ये सब बातें जो हो रही हैं, कैसे होती हैं, बताइये?

IS THERE ANYTHING LIKE HYPNOTISM, TELEPATHY, OR THE OCCULT GOING ON? WHAT IS THE EXPLANATION?

✓ क्या ऐसे लगता है जैसे कोई जादू टोना हो रहा है, ओपरी कसर या भूत-प्रेतों का असर है? यह सब कैसे होता है?

INCLUDE DELUSIONAL EXPLANATIONS IN TERMS OF PARANORMAL PHENOMENA : e.g. hypnotism, telepathy, magic, witchcraft, etc. Exclude purely subcultural beliefs; → 83.

- 1 = Partial delusions
2 = Full delusions

IS ANYTHING LIKE ELECTRICITY, OR RADIO-WAVES AFFECTING YOU?

✓ क्या ऐसे लगता है कि बिजली, एक्सरे या मशीनों का आप पर असर हो रहा है?

(In what way? What is the explanation?)

INCLUDE DELUSIONAL EXPLANATIONS IN TERMS OF PHYSICAL FORCES : e.g. radio, television, X-rays, electricity, transmitters, microphones, machines of various kinds.

- 1 = Partial delusions
2 = Full delusions

DELUSIONS OF ALIEN FORCES PENETRATING OR CONTROLLING MIND (OR BODY).

Include any delusion, whether rated elsewhere or not, which involves an external force penetrating the subject's mind or body, e.g. rays turn liver to gold, alien thoughts pierce skull or are inserted into mind, hypnotism makes patient levitate, a spirit speaks with subject's voice, a radio transmitter has been implanted into brain and broadcasts thoughts or controls actions, etc.

- 1 = Partial delusions
2 = Full delusions.

☐ (76)

☐ (77)

☐ (78)

☐ (79)

☐ (80)

☐ (81)

CHOOSE, LIKELY DELUSION, AND ASK :

HOW DID IT COME INTO YOUR MIND THAT THIS WAS THE EXPLANATION?

✓ आपको कैसे पता लगा कि यही कारण है?

(Did it happen suddenly? How did it begin?)

RATE PRIMARY DELUSIONS : *Based upon experiences in which subject suddenly becomes convinced that a particular set of events has a special meaning. (See definition on page 26.) Not based on mood or explanation of other abnormal experiences.*

☐ (82)

- 1 = Partial delusions.
2 = Full delusions

15F OTHER DELUSIONS

(Examiner should question as appropriate)

RATE SUBCULTURALLY INFLUENCED DELUSIONS : *Include only subjects who belong to small groups with definitely idiosyncratic beliefs; small sects tribes, 'secret societies', etc.*

☐ (83)

- 0 = No significant subcultural influence. For example, an English subject believing he is influenced by TV would be rated (0) since, although the delusion depends on TV being available in England, it is not in any way specific to a small subcultural group.
- 1 = One or more of the 'delusions' rated earlier could easily be no more than a belief shared by other members of the subject's subcultural group, i.e. the Pentecostal church with the gift of tongues. Voodoo, witchcrafts, communicating with God, are other examples of beliefs which may be taken quite literally by groups of people who are not clinically deluded, Rate (1) if subject holds such beliefs without elaborating them further.
- 2 = As (1), but because of excitement, expansiveness, depression, confusion, intellectual retardation, etc., the subject holds the beliefs with exceptional fervour and conviction, or elaborates them further. Such a subject might well be regarded as abnormal by other members of his own sect or group.
- 3 = More specific delusional states, e.g. Koro, Witigo, etc.

(Do you have any reason to be jealous of anybody?)

MORBID JEALOUSY.

- 1 = Partial delusions
2 = Full delusions

☐ (84)

DELUSION OF PREGNANCY.

- 1 = Partial delusions
2 = Full delusions

☐ (85)

SEXUAL DELUSIONS : *Any delusion with sexual content, e.g. fantasy lover, sex changing, etc. Do not include an untrue claim that a subject is married or has children.*

- 1 = Partial delusions
2 = Full delusions

☐ (86)

HAVE YOU HAD ANY UNUSUAL EXPERIENCE OR ADVENTURES RECENTLY?

✓ क्या इन दिनों आपको कोई खास अनुभव (तजुबा) हुआ या कोई अजीब घटना (बात) हुई?

RATE FANTASTIC DELUSIONS, DELUSIONAL MEMORIES, DELUSIONAL CONFABULATIONS, FANTASTIC DELUSIONS :

☐ (87)

Confabulation : Subject makes up delusions on the spot. Very rare.

Delusional memories : Subject seems to be describing actual memories.

) *Describes the same delusions time and again. Not confabulations. Rare, e.g. 'I Came down to earth on a silver star.' Fantastic delusions : The commonest of the three, e.g. England's coast melting.*

1 = Partial delusions

2 = Full delusions

3. SIMPLE DELUSIONS BASED ON GUILT, DEPERSONALISATION, HYPOCHONDRIASIS, ETC.

Definition

) *These symptoms often appear to be based on a depressed mood and are relatively consistent and unelaborated. Do not include more bizarre elaborations of any of them, e.g. having a metal nose = symptom 87, not 89. Having been turned into another specified person = possibly symptom 71, not 90. Liver turned to lead by X-rays = symptoms 80 and 81, not 91. England's coast melting = symptom 87, not 92.*

DO YOU FEEL YOU HAVE COMMITTED A CRIME, OR SINNED GREATLY, OR DESERVE PUNISHMENT?

✓ या आपको ऐसा महसूस होता है कि आपने कोई बड़ा पाप या अपराध किया है, जिसके लिये आपको सज़ा मिलनी ही चाहिये?

(Have you felt that your presence might contaminate or ruin other people?)

RATE DELUSIONS OF GUILT.

1 = Subject has brought ruin to family by being in present condition, or thinks that symptoms are a punishment for not doing better, etc. Does not elaborate as in (2).

2 = Subject says has sinned greatly or committed some terrible crime or brought ruin upon the world. May feel deserving of punishment, even of death or hell-fire, because of it.

(Do you think your appearance is normal?)

RATE SIMPLE DELUSIONS CONCERNING APPEARANCE :

(Nose too large, teeth misshapen, body crooked, etc.)

1 = Strong feelings that there is something wrong with appearance; subject looks old or ugly or dead, skin cracked, teeth misshapen, nose too large, body crooked, etc. Can be reassured temporarily. There may be only one limited preoccupation.

2 = Subject acts accordingly (plastic operations, etc.)

(Is anything the matter with your brain?)

RATE DELUSIONS OF DEPERSONALISATION : *Subject has no head, does not exist, hollow instead of a brain, etc.*

1 = Unable to think, no thoughts in head, feels as though he has no brain or as though it does not function at all.

2 = Symptom more intense. Subject has no head, no brain, does not exist.

☐ (88)

☐ (89)

☐ (90)

(Is anything the matter with your body?)

RATE HYPOCHONDRIACAL DELUSIONS : *Subject has incurable cancer, bowels are stopped up, insides are rotting, etc.*

☐ (91)

- 1 = Subject feels body is unhealthy, rotten, diseased, but without the force of (2).
- 2 = Subject has incurable cancer, bowels are stopped up or rotting away, etc.

(Do you have the feeling that something terrible is going to happen? What?)

RATE DELUSIONS OF CATASTROPHE : *World is about to end, some catastrophe has happened or will occur, everything is evil and will be destroyed.*

☐ (92)

- 1 = Subject feels sense of impending doom; something awful will happen. Non-specific but out of proportion to circumstances.
- 2 = Delusional conviction that world is about to end or some other enormous catastrophe is about to occur or has occurred. World is dirty, decayed, rotten; i.e. further delusional elaboration of (1).

15H. GENERAL RATINGS OF DELUSIONS AND HALLUCINATIONS.

(Include both partial and full delusions.)

CONSIDER BOTH DELUSIONS AND HALLUCINATIONS IN FOLLOWING RATINGS

RATE SYSTEMATISATION OF DELUSIONS.

☐ (93)

Scoring :

- 0 = No delusions or hallucinations.
- 1 = Delusions and hallucinations not elaborated into a general system affecting much of the subject's experience. Include encapsulated delusions or isolated hallucinations.
- 2 = Some systematic elaboration, but substantial areas of the subject's experiences are not affected.
- 3 = Subject interprets practically all his experience in delusional terms.

RATE EVASIVENESS

☐ (94)

Scoring :

- 0 = No attempt at concealment suspected.
- 1 = Examiner suspects that there may be (either) delusions or hallucinations in the background, but the subject is not concealing much of the psychopathology.
- 2 = Examiner suspects that there is a considerable preoccupation with delusions (even a delusional system) or hallucinations, but the subject tries to conceal them.
- 3 = No concealment but other delusions or hallucinations probably present. Not elicited because of poor intelligence and education or incoherence or muteness, etc.

OVERALL RATING OF PREOCCUPATION WITH DELUSIONS AND HALLUCINATIONS.

☐

(95)

Scoring :

- 0 = No delusions or hallucinations.
- 1 = No delusions or hallucinations definitely reated but examiner suspects that they may be present.
- 2 = Preoccupied with past delusions or hallucinations only. Not actively deluded or hallucinated at present.
- 3 = Delusions or hallucinations definitely present but subject is not preoccupied with them for much of the time. Can turn attention to other things without difficulty.
- 4 = Delusion is or hallucinations present and take up most of the subject's attention. Preoccupied to the exclusion of many other matters.
- 5 = Patient can hardly discuss anything but delusions.

RATE ACTING OUT DELUSIONS

(Rate from case-record, etc.)

☐

(96)

Scoring :

- 0 = No delusions or hallucinations.
- 1 = Subject able to keep delusions or hallucinations to himself, or to confide them only to a few trusted people (sympathetic relatives, friends, doctors, etc.). He does not express them in public nor act upon them. Does not talk out loud to voices.
- 2 = Subject has acted upon delusions or hallucinations during past month, or expressed them in public (i.e. outside the small circle of people who would be expected to be sympathetic). This has not however resulted in severe social disturbance or a social crisis.
- 3 = As (2) but acting out, or public expression, has resulted in severe social disturbance or a social crisis.

16. SENSORIUM AND FACTORS AFFECTING

** HAVE YOU HAD ANY LAPSES OF MEMORY RECENTLY?

क्या इन दिनों याददाश्त में कमी (कमजोरी) महसूस हुई?

(Have there been any periods in which you completely forgot what happened?)

(What was it like?)

(How do you explain it?)

RATE FUGUES, BLACKOUTS, AMNESIA LASTING MORE THAN ONE HOUR : *irrespective of aetiology.*

☐

(97)

0 = less than 12 hours.

1 = 12—24 hours.

2 = more than 24 hours.

** WHAT MEDICINES OR DRUGS DO YOU TAKE?

आजकल आप कौन सी दवाईयां ले रहे हैं?

(Do you take anything for your nerves or your mood?)

(Obtain list of drugs.)

(Who prescribes?)

1)

2)

93

(94

RATE DRUG ABUSE DURING MONTH. *One category only.*

- 1 = Cannabis.
- 2 = Amytal, etc.
- 3 = LSD, amphetamine, etc.
- 4 = Cocaine, heroine, etc.

☐ (98)

**** MAY I ASK ABOUT YOUR DRINKING HABITS? HOW MUCH DO YOU USUALLY DRINK EACH DAY?**

17

क्या आप शराब पीते हैं? औसतन एक दिन में कितनी पी लेते हैं?

(Is alcohol in any way a problem for you? In what way?),

(CHECK LIST : *Present on card if needed.* During the past month have you:
 had family problems because of drinking?
 missed work because of drinking?
 had morning shakes or other withdrawal symptoms?
 had blackouts for several hours?
 heard voices or seen visions?)

RATE ALCOHOL ABUSE DURING PAST MONTH.

- 1 = Agrees alcohol has been a problem but not 2.
- 2 = Any check-list item applies.

☐ (99)

RATE DISSOCIATIVE STATES DURING PAST MONTH :

'Narrowing of consciousness which serves an unconscious purpose and is commonly accompanied or followed by a selective amnesia, e.g. trance, possession state, fugue, hypersomnia, stupor, etc. Do not include if caused by drugs, alcohol, epilepsy, etc.

- 1 = Present during the past month, but not at examination.
- 2 = Present at examination.

☐ (100)

RATE CONVERSION SYMPTOMS, e.g. paralysis, anaesthesia, blindness, tremor, seizures, etc. if mentioned during interview.

- 1 = Present during month, not at examination
- 2 = Present at examination.

☐ (101)

RATE CLOUDING OR STUPOR AT EXAMINATION

- 1 = Clouding : Inadequate comprehension of external impressions, with perplexity, and impairment of attention and orientation.
- 2 = Stupor : Subject appears comatose but there is no clouding or impairment of consciousness.

☐ (102)

IF ANY SUSPICION OF POOR MEMORY OR DISORIENTATION :

MAY I ASK ONE OR TWO STANDARD QUESTIONS WE ASK OF EVERYBODY?

क्या मैं एक दो ऐसे सवाल पूछ सकता हूँ जो कि हम सब से पूछते हैं?

HOW OLD ARE YOU?

आपकी उम्र कितनी है?

CAN YOU TELL ME THE YEAR AND THE MONTH?

क्या आप बता सकते हैं कि आजकल कौन सा महीना चल रहा है, और कौन सा साल?

WHAT IS THE NAME OF THE PRIME MINISTER?

हमारे प्रधान मंत्री का क्या नाम है?

RATE ORGANIC IMPAIRMENT OF MEMORY. See glossary for definition.

- 1 = Mild.
2 = Moderate.
3 = Severe.

☐

(103)

17

17. INSIGHT

**

** DO YOU THINK THERE IS ANYTHING THE MATTER WITH YOU?

क्या लगता है कि आपको कोई बीमारी है या आपमें कोई नुक्स है?

(What do you think it is?)
(Could it be a nervous condition?)
(What do you think the cause is?)
(Why did you need to come to hospital?)
(Do you think (specify delusions or hallucinations) were part of a nervous condition?)

IF PSYCHOTIC SYMPTOMS (i.e. SYMPTOMS FROM SECTIONS 12-15):

- 0 = Full insight (intelligent subject, able to appreciate the issues involved).
1 = As much insight into the nature of the condition as social background and intelligence allow.
2 = Agrees to a nervous condition but examiner feels that subject does not really accept the explanation in terms of a nervous illness (e.g. gives delusional explanation, the result of persecution, or rays, etc.)
3 = Denies nervous condition entirely.
9 = Psychotic illness not present.

☐

(104)

IF NEUROTIC SYMPTOMS (i.e. SYMPTOMS FROM SECTION 1-11 ONLY):

- 0 = Full insight (intelligent subject able to appreciate the issues involved).
1 = As much insight into the nature of the condition as social background and intelligence allow.
2 = Gives physical explanation for neurotic symptoms.
3 = Denies neurotic symptoms entirely.
9 = Neurotic illness not present.

☐

(105)

**

** OF ALL THE PROBLEMS YOU HAVE TOLD ME ABOUT, WHICH ONE AFFECTS YOU MOST?

जो भी परेशानियां आपने मुझे बताई है, उनमें से सबसे ज्यादा तकलीफ आपको किस से हैं?

HOW MUCH DOES IT INTERFERE WITH YOUR WORK OR YOUR RELATIONSHIPS WITH OTHER PEOPLE?

इन सब का आपके कामकाज, दूसरों के साथ मेल जोल पर कितना असर पड़ा है?

(Have you actually been out of work, or been unable to do the housework or go shopping, travelling, etc. during the past month?)
(Have the symptoms impaired your efficiency in any other way?)

RATE SOCIAL IMPAIRMENT DUE TO NEUROTIC CONDITION.

☐ (106)

- 0 = No neurotic or psychotic symptoms present.
- 1 = Neurotic symptoms present but little diminution of subject's efficiency or interference with everyday activities.
- 2 = Neurotic symptoms interfere with subject's efficiency to moderate extent but are not incapacitating. e.g. subject neglects housework or can't enjoy leisure activities or social relationships, or finds work efficiency reduced because of worry, tension, irritability, depression, anxiety, etc. Subject does not, however, stop work altogether or completely neglect household.
- 3 = Subject severely incapacitated by neurotic symptoms: had to have at least a week off work during past month; was housebound for a week or more; was actively withdrawn from all social relationships, etc. The subject does not have to be totally incapacitated for the whole month for this rating to be made, but impairment has to be very severe.
- 8 = Examiner unsure.
- 9 = Psychotic condition present.

(If both psychotic and neurotic condition, rate whichever shows more impairment.)

RATE SOCIAL IMPAIRMENT DUE TO PSYCHOTIC CONDITION

☐ (107)

- 0 = No neurotic or psychotic symptoms present.
- 1 = Psychotic symptoms present but little diminution of subject's efficiency or interference with everyday activities.
- 2 = Psychotic symptoms interfere with subject's efficiency to a moderate extent but are not incapacitating. e.g. subject neglects housework or can't enjoy leisure activities or social relationships, or finds work efficiency reduced. Subject does not, however, stop work altogether or completely neglect household.
- 3 = Subject severely incapacitated by psychotic symptoms: had to have at least a week off work during past month; was housebound for a week or more; was actively withdrawn from all social relationships etc. The subject does not have to be totally incapacitated for the whole month for this rating to be made, but impairment has to be very severe.
- 8 = Examiner unsure.
- 9 = Neurotic conditions, and no psychotic condition, present.

FINAL QUESTION :**** HAVE THERE BEEN ANY OTHER THINGS LATELY THAT I HAVEN'T COVERED?**

क्या इन बिन्दुओं आपकी और भी तकलीफें रही हैं जिन के बारे में मैंने आपसे अभी तक कुछ नहीं पूछा?

Specify :

Note here any points that seem to be important or unusual about the subject or the interview which are not covered in the schedule.

Reconsider schedule to make sure that all obligatory questions have been asked. Also consider whether behaviour, affect and speech ratings can be made or whether further observation or examination is necessary. IF NOT, THIS IS THE END OF THE INTERVIEW.

18-20 BEHAVIOUR, AFFECT AND SPEECH RATINGS

- 0 = Symptom absent.
 1 = Present in fairly severe degree, or very severe but intermittent during interview.
 2 = Present in very severe degree and almost continuous during interview.
 8 = Examiner not sure.
 9 = Subject not examined, or examination not appropriate.
 N. B. If in doubt, rate (0). A rating of (1) means there is no doubt about the symptom being present in fairly severe form.

Behaviour during interview :

Self-neglect (cleanliness, shaven, make-up, state of hair and clothes).

☐ (108)

Bizarre appearance (secret documents openly displayed, special clothes or ornaments with symbolic significance, etc. Do not include mannerism or posturing = symptom 116).

☐ (109)

Slowness and underactivity (sits abnormally still, walks abnormally slowly, delay in performing movements).

☐ (110)

Agitation (fidgety, restlessness, pacing, frequent unnecessary movements).

☐ (111)

Gross excitement and violence (throws things, runs or jumps about, waves arms wildly, shouts or screams).

☐ (112)

Irreverent behaviour (sings, facetious, silly jokes, flippant remarks, unduly familiar).

☐ (113)

Distractibility (stops talking or changes subject due to distraction by trivial noises, or events outside the room or turns attention to furniture, etc.)

☐ (114)

Embarrassing behaviour (making sexual suggestions or advances to interviewer; loss of social restraint—scratches genitals, passes loud flatus, etc.)

☐ (115)

Mannerisms and posturing (odd, stylised movements or acts, usually idiosyncratic to the patient, often suggestive of special meaning or purpose: assuming and maintaining uncomfortable or inappropriate postures).

☐ (116)

Stereotypies, etc. (constant repetition of movements or postures such as, rocking, rubbing, nodding, grimacing: no special significance).

☐ (117)

Behaves as if hallucinated (non-verbal: as though hears voices or visions: lips move soundlessly, looks round, giggles to self—not just from embarrassment, shyness, etc.)

☐ (118)

Catatonic movements

☐ (119)

(Negativism: does the opposite of what he is asked.

Ambitendence: begins to take proffered hand, then withdraws; etc.

Echopraxia: imitates examiner's movement.

Flexibilitas cerea: arm remains where it is put, for at least 15 seconds.

Mitgehen: excessive co-operation in massive movement.

Echolalia: imitates words and phrases with same intonation and inflection of voice.)

(These items can be separately rated in special projects.)

Affect during interview

Observed anxiety (tense worried look or posture, fearful apprehensive look, frightened tone of voice, tremor).

☐ (120)

Observed depression (sad, mournful look, tears, gloomy tone of voice, deep sighing, voice chokes on distressing topics).

☐ (121)

Histrionic (feelings expressed in exaggerated, dramatic, histrionic manner).

☐ (122)

Hypomanic affect (unduly cheerful, smiling, euphoric, elated.)

Hostile irritability (unco-operative, irritable, angry, overtly hostile, discontented, haughty, antagonistic).

Suspicion

Perplexity (puzzlement).

Lability of mood (whether lability of one mood, or changing from one mood to another).

Blunted affect (expressionless face and voice, uniform blunting whatever the topic of conversation, indifference to distressing topics, whether delusional or normal).

1 = Blunting not uniform, e.g. at times responds affectively but at other times is markedly flat; or responds with some evidence of affect, but definitely less than expected.

2 = Severe and uniform blunting.

Incogruity of affect (emotion is shown, but not congruent with topic).

Speech during interview

Slow speech (long pauses before answering, long pauses between words).

Pressure of speech (more copious speech than normal, too rapid speech, very loud voice, too circumstantial speech.)

Non-social speech (talks, mutters, whispers out loud, out of context of conversation with examiner).

muteness

1 = Almost mute, fewer than twenty words in all.

2 = Totally mute.

Restricted quantity of speech (subject frequently fails to answer, questions have to be repeated, restricted to minimum necessary, no extra sentences, no additional comments).

Neologisms and idiosyncratic use of words or phrases, e.g. One is called "Per-God" and the other is called "Per-the-devil".

"... miracle-willed through God's 'tarn-harn' ...", 'Well, there is a frequenting of clairvoyance ...' 'Per-God, Devil' and 'tarn-harn' are neologisms; 'frequenting of clairvoyance' is an example of ordinary words used idiosyncratically. DO NOT RATE THIS SYMPTOM PRESENT UNLESS EXAMPLES ARE WRITTEN DOWN.

Disorder of content of speech.

Three types of disordered content are specified: in each case, the effect is to make it very difficult to grasp what the subject means. However, the symptoms are defined in terms of specific components so that it should, in most cases, be possible to say Whether one, two or all three symptoms are present. If in doubt, rate hierarchically, i.e. rate incoherence in preference to flight of ideas in preference to poverty of speech.

If the patient does not talk enough to give a rateable sample of speech, rate all three symptoms Y.

Incoherence of speech. The subject's meaning is obscured by distorted grammar, lack of logical connection between one part of a sentence and another or between sentences, sudden irrelevances or 'Knight's move', grossly pedantic phrases, answering of the point, etc. For example:

'We've seen the downfall of the radium crown by the Roman Catholics, whereas when you come to see the drinking side of the business, God saw that Noah, if he lost his reason, he got nobody there to look after them.'

'I did suggest to you, that intrinsic or congenital sentiment or refinement of disposition would be so miracle-willed through God's 'tarn-harn' to assume quite the opposite.'

☐ (123)

☐ (124)

☐ (125)

☐ (126)

☐ (127)

☐ (128)

☐ (129)

☐ (130)

☐ (131)

☐ (132)

☐ (133)

☐ (134)

☐ (135)

☐ (136)

"I believe we live in a world, in an age, where the elements are a force that elders of professionalism hope, not to conquer, but to control."

"What's your address?" "Its supposed to be salisbury near Birmingham" (Vorbereiden.)

DO NOT RATE THIS SYMPTOM PRESENT UNLESS EXAMPLES ARE WRITTEN DOWN.

A rating of 2 means that very little normal speech is present.

N. B. A free flow of delusions is not necessarily incoherent. A subject may talk about delusions quite coherently.

Flight of Idea. Words are associated together inappropriately by sound or rhyme (clang association). Although the original aim of the sentence may quickly be lost, a path can be traced through associations of the white-black-coffin or ring-wrong variety, or through associations with distracting stimuli, e.g.

☐ (137)

"How is your appetite?" "I feel as if I have lost my appetite. I have had an orange. A real juicy orange." (See patient walking past window.) "She is going for E.C.T. Etcetera treatment or teddy bear's picnic. I call it."

DO NOT RATE THIS SYMPTOM PRESENT UNLESS EXAMPLES ARE WRITTEN DOWN.

A rating of 2 means that very little normal speech is present.

Poverty of content of speech. The subject talks freely but so vaguely that little information is given in spite of the number of words used: rambles on without coming to a point; may wander far from original theme. Exclude incoherence or flight of ideas. Rate only if severe and always give written example.

☐ (138)

Misleading Answers Subject's answers are misleading because answers 'yes' or 'no' to everything, or frequent self-contradictions, or appears to be deliberately misleading. Do not include incoherence, flight of ideas or poverty of speech here.

☐ (139)

Re-rate adequacy of interview

- 0 = Ratings made adequately represent the symptoms present.
- 1 = Some problem but key symptoms have been rated.
- 2 = Serious questions as to adequacy of interview for rating key symptoms (other than sections 18-20).
- 3 = Only sections 18-20 could be rated.

☐ (140)

Check that every box has an entry except those below ticked cut-off points.

Complete coding sheet if one is being used.